# KinPath: Year-One Business Plan and Market-Cornering Strategy

## 1. Executive Summary

The original business plan for this app identified the core strategic insight: nobody has successfully monetized this product by charging grieving families a subscription, and every profitable or fast-scaling competitor (Empathy, Trust & Will, Everest) built on enterprise distribution. This document answers the four follow-up questions: what additional features are missing from the current app design that would help corner the market, what the first 12 months of operation should look like, what the app should be named, and how to make it genuinely enticing to employers, hospices, hospitals, and other paying partners.

The throughline of this plan is a repositioning. The app is not best described as a "funeral app" — it is better positioned as the **next-of-kin operating system**, the trusted software layer that employers, hospices, hospitals, and insurers hand to families at the moment of loss, covering everything from disposition coordination through the one-year anniversary. That framing gives a bigger category than funeral planning alone and makes every B2B pitch stronger, because a "kin OS" is something an institution can own and distribute as its own benefit rather than a third-party marketplace.

The recommended working name is **KinPath**. It signals family, guidance, and the journey without using the words death, funeral, or grief in the brand itself — a critical filter given that bereaved users are in a state of heightened emotional sensitivity and will react negatively to anything clinical or morbid in the first impression.

## 2. App Name: KinPath

The recommended working name is **KinPath** (working name — pending trademark clearance and domain availability).

Rationale:

- **"Kin"** signals family and next-of-kin, the exact role the app is built around. It is warm, short, universally understood, and inclusive of chosen family and non-traditional households.
- **"Path"** signals guidance, a journey, and a step-by-step process — which is literally what the app delivers: a guided path through every task from death through the one-year anniversary.
- The combination avoids the words "death," "funeral," "grief," "died," "departed," "eternal," and "rest" — all of which are either morbid, clinical, or religiously specific. A bereaved person seeing the name in a push notification from their employer's benefits portal should feel cared for, not confronted.

- The name is brandable, pronounceable, and short enough for an app icon, a subdomain, and a partner's co-branded benefits page (e.g., "MetLife, powered by KinPath").

Alternative names considered:

| Name | Strengths | Weaknesses | Verdict |
|---|---|---|---|
| **KinPath** (recommended) | Warm, short, journey-implying, avoids morbidity | None significant | Final pick |
| AfterGuide | Clear sequencing, descriptive | "After" implies the person is gone, slightly cold | Strong backup |
| Kindred Path | Evocative, premium | Longer, harder to fit in app icon | Second choice |
| NextKin | Direct, references next-of-kin | Sounds like a legal term, slightly transactional | Reject |
| Hearthside | Very warm, domestic | Does not signal guidance or process | Reject |
| OneLessThing | Emotional relief framing | Hard to build a brand around, not professional for B2B | Reject |
| Wayfare | Elegant, modern | Vague, no kin signal | Reject |
| Bereave | Already taken (existing competitor Bereave.io) | Trademark conflict | Reject |

A trademark search and .com domain availability check should be the first action after this plan is approved, with the domain purchased and a brand-squatting defense filed immediately. If KinPath is unavailable, the fallback is **AfterGuide**, with **Kindred Path** as the third choice.

## 3. What Is Missing: Market-Cornering Features

The original app design covered: state- and religion-specific checklists, vendor marketplace (funeral homes, florists, caterers), a local funeral coordinator, legal/financial resource directories, a document vault, and a 4–5 person family permission model. Those are the table-stakes features. To genuinely corner the market — meaning to become the default platform that no insurer, employer, or hospice can launch a bereavement benefit without — the following additions are needed.

### 3.1 The Benefits Recovery Engine (the single most valuable missing feature)

The app should not just list legal and financial resources — it should actively track and help recover money that families may be eligible for but do not always claim. This is the feature that turns the app from a helpful checklist into something that can pay for itself many times over, which is the exact pitch every B2B buyer needs.

Many deaths leave benefits, reimbursements, or administrative value unclaimed:

- The Social Security lump-sum death benefit is $255, but in 2023 it resulted in a payment in fewer than 38% of insured-worker deaths; the benefit may be missed if families do not know it exists or if they miss the two-year filing deadline (SSA Pub. 05-10008; CRS R43637).

- VA burial benefits of $2,000 for service-connected deaths or $1,002 plus a $1,002 plot allowance for non-service-connected deaths may be missed if families do not know the veteran's service history qualifies them (VA burial allowance; VA Survivor Benefits Booklet).
- Life insurance policies can be overlooked when families do not know a policy exists.
- 401(k) and pension beneficiary claims require a death certificate and proof of relationship, and can be delayed if required documents are not ready (IRS, Retirement Topics – Death).
- Employer-provided group life insurance, accidental death benefits, and travel insurance benefits can be overlooked if the family does not contact the employer's HR/benefits team.

The Benefits Recovery Engine would:

1. Inventory every potential benefit source based on the deceased's profile (employment status, military service, insurance holdings, retirement accounts, union membership).
2. Generate pre-filled claim packets with checklists of required documents for each.
3. Track deadlines (SSA's two-year window, VA's two-year window, state life-insurance prompt-payment statutes).
4. Surface a running "estimated recoverable value" figure — a dollar amount the app has helped identify and pursue — that becomes the headline ROI number in every B2B pitch.

This is the feature that makes KinPath a quantifiable benefit rather than a soft emotional one, which is the single most important lesson from Empathy's success: Empathy sells a documented 35% reduction in claims-operations costs to insurers, not an emotional appeal ( Empathy).

### 3.2 Family Governance and Conflict-Reduction System

The original plan mentioned limiting access to 4–5 people, but to genuinely reduce the "major element of family feuds and chaos" the user specifically named, the app needs a structured governance layer:

- **A designated Coordinator role** (the primary account holder) with sole authority to assign and revoke helper access, set per-field permissions, and serve as the single source of truth for decisions.
- **An immutable decision log** — every choice the family makes (disposition type, vendor selected, service date, who pays for what) is recorded with a timestamp and the person who made it. This is the single most powerful anti-feud feature: when a dispute arises weeks later, the record shows who decided what and when, rather than relying on conflicting memories during grief.
- **A shared expenses ledger** with expense splitting — who paid for the casket, who paid for catering, who is owed reimbursement — so money, the most common source of post-death family conflict, is transparent and auditable rather than negotiated in group texts.
- **A "raise an objection" mechanism** for major decisions (disposition, service location, headstone) that requires all permissioned helpers to acknowledge before the decision is finalized, creating a lightweight consensus process that prevents one relative from being blindsided.

- **Optional exportable record** — the decision log and expense ledger can be exported as a structured document that can be shared with an attorney, mediator, or court if a dispute escalates. Having a clear, timestamped record of decisions and who made them is often the single most effective way to resolve a family disagreement before it becomes a legal dispute.

### 3.3 The Coordinator Network (Death Doulas as the Local Service Layer)

The original plan referenced "a funeral coordinator within their area." To corner the market, KinPath should not just match families to existing coordinators — it should build and own a network of vetted death doulas and family coordinators, a vetted local coordinator marketplace layer that does not yet exist as a branded network.

This is feasible specifically because death doulas have no state licensure requirement anywhere in the United States (NEDA FAQ; IEOLCA), and two states — California and Indiana — have specifically blocked funeral boards from imposing funeral-director licensure requirements on them (Institute for Justice; Bloomberg Law). This means KinPath can recruit, train, certify to its own internal standard, and deploy coordinators nationwide without navigating 50 separate state licensing regimes — a structural advantage no licensed funeral home can match.

The coordinator network becomes a vetted local coordinator marketplace: families get a vetted human guide, and doulas get a software platform, a referral pipeline, and standardized tooling that lets them scale beyond the 1–2 families at a time most can serve today. KinPath takes a coordination fee on each match, creating a revenue stream that scales with network density.

### 3.4 Partner-Facing Dashboards (the B2B lock-in mechanism)

This is the feature that makes KinPath indispensable to every institutional buyer. Each partner type gets a dedicated dashboard:

- **Employer dashboard:** shows aggregate, de-identified utilization (how many employees activated KinPath, average time-to-resolution, satisfaction scores) and case-management workflow for HR — tracking leave status, return-to-work scheduling, and manager check-in reminders. This directly fills the gap that EAPs are documented as not filling: EAPs do not train managers on grief conversations, do not coordinate leave, and do not create return-to-work plans (Bereave; Bereave).
- **Hospice dashboard:** tracks the federally mandated bereavement follow-up obligation (up to 12 months post-death under 42 CFR §418.64) with automated outreach, contact logging, and surveyor-ready compliance reports (CMS State Operations Manual, Appendix M; 42 CFR §418.64). This is the killer feature for hospices: the app converts an unfunded federal mandate into a turnkey, auditable compliance program.
- **Hospital decedent-affairs dashboard:** gives the hospital's Office of Decedent Affairs a workflow tool for the exact functions those teams already perform — coordinating disposition, meeting families, liaising with funeral homes and coroners (OhioHealth; Duke Health; Stanford ODA). Among surveyed decedent-affairs programs, the only universal

function was answering family questions — a software tool that structures and scales that function is an immediate value-add.

- **Insurer dashboard:** tracks beneficiary claims, documents submitted, and time-to-payment, giving the carrier the same claims-cost-reduction story Empathy sells but with a broader scope (estate coordination, not just grief support).

The dashboards are what create switching costs: once a hospice has a year of bereavement compliance data inside KinPath, or an employer has integrated its leave workflow, migrating off the platform is painful. That is the moat.

### 3.5 Identity and Estate Protection Layer

Death is the single highest-risk period for identity theft, because a deceased person's SSN, date of birth, and address are now permanently unmonitored. The app should include an automated identity-protection workflow:

- One-tap credit bureau notifications (Equifax, Experian, TransUnion) to seal the deceased's credit file (Equifax; TransUnion; Experian).
- Fraud monitoring alerts if new accounts are opened in the deceased's name.
- A "digital afterlife" module: deactivating social media accounts, email, online subscriptions, and loyalty programs — a task set that is genuinely painful and that no competitor currently owns end to end.
- A checklist for preventing obituary-driven scams (burglary during funeral services, fake charity solicitations) which the FBI has repeatedly warned about.

### 3.6 Pre-Need Mode (the expansion into estate planning)

The original plan focused on at-need (post-death). To corner the market, KinPath should add a pre-need mode that lets users pre-organize their own end-of-life preferences, documents, and beneficiary instructions before a crisis. This does several things strategically:

- It creates a recurring engagement surface (not just a one-time crisis tool), which justifies a subscription for the pre-need user and gives KinPath a relationship with the eventual coordinator before the death even occurs.
- It positions KinPath against Trust & Will and Everplans in the estate-planning market rather than just against funeral-specific tools — a larger, higher-value category.
- It generates a pre-need data asset: when a pre-need user eventually passes, KinPath already holds their preferences, documents, and designated coordinator, meaning the at-need family's onboarding is near-instant instead of starting from zero. This is a genuine structural advantage no at-need-only competitor can replicate.

### 3.7 Partner APIs and Integrations

To become the platform rather than a point solution, KinPath needs open APIs into:

- **HRIS systems** (Workday, BambooHR, ADP) so employer activation is a one-click benefit enrollment, not a manual referral.
- **EAP platforms** (Optum, Spring Health, Lyra) so KinPath can be the bereavement module inside an existing mental-health benefits stack rather than competing with it.
- **Life insurance claims systems** so the Benefits Recovery Engine can file and track claims programmatically with partner carriers.
- **Funeral home management software** (Funeralocity, Halcyon) so vendor listings stay current without manual upkeep.
- **Estate planning platforms** (Trust & Will) via partnership rather than building will-drafting in-house, which carries unauthorized-practice-of-law risk.

### 3.8 Grief Support With a Clinical Layer

The original plan included grief resources, but to be defensible to insurers and employers, KinPath should integrate licensed grief counseling as an optional escalation path — either by employing or contracting licensed clinical social workers, or by partnering with a teletherapy network. This matters because:

- It lets the app make a clinical outcomes claim (reduced depression, faster return to work) that pure administrative tools cannot, which is what employers are willing to pay PEPM for.
- It matches the Empathy model, which combines emotional support with administrative task management as a single product (Empathy).
- It must be scoped carefully: grief support and psychoeducation are not the unauthorized practice of medicine or psychology, but clinical counseling requires licensed providers. The app should be the front door that routes to licensed care, not the care provider itself, unless KinPath deliberately builds a clinical division.

## 4. Market-Cornering Strategy

The strategy is to win a category, not a feature set. The category is "next-of-kin operating system for institutions and families" — the software layer that every organization touching end-of-life eventually needs.

### 4.1 The Wedge

The initial wedge is the **hospice bereavement obligation**, not the consumer funeral purchase. Every Medicare-certified hospice is legally required to provide up to 12 months of bereavement support to surviving family, but this requirement generates no dedicated reimbursement and is widely treated as an unfunded compliance burden (CMS State Operations Manual, Appendix M; 42 CFR §418.64). KinPath converts that mandate into a turnkey, surveyor-ready compliance

program — a pitch that sells itself because the buyer is already spending uncompensated staff time on exactly this task.

This wedge is far more defensible than a consumer funeral marketplace because:

- The buyer (hospice) has an existing legal obligation, not a discretionary purchase.
- The buyer is identifiable and finite (roughly 3,700 Medicare-certified hospices in the U.S.), making direct sales feasible.
- The buyer's switching cost, once a year of compliance data lives in KinPath, is high.
- No competitor currently owns this channel — the only hospice partnerships found in prior research dated to 2021 pilots that never scaled (Hospice News; Hospice News).

### 4.2 The Moats

1. **Network effects in the coordinator network.** As KinPath signs more doulas, families get faster matches and better coverage; as KinPath serves more families, doulas get more work, creating a two-sided flywheel that is hard for a new entrant to replicate.
2. **Data advantage in the Benefits Recovery Engine.** Over time, KinPath accumulates the most comprehensive dataset of which benefits go unclaimed, by demographic and geography, which lets it continuously improve recovery rates — a compounding data moat no point-solution competitor can match.
3. **Compliance lock-in via dashboards.** Once a hospice or employer has a year of operational data inside KinPath, switching means re-creating that compliance history from scratch — a genuine switching cost.
4. **Pre-need to at-need data continuity.** Users who pre-organize through KinPath create a data asset that makes their family's eventual at-need onboarding near-instant — an experience advantage that only a platform spanning both phases can deliver.
5. **Regulatory positioning as the compliant default.** By building to SOC 2 Type II, GLBA Safeguards Rule, and FTC Health Breach Notification Rule standards from day one (FTC Safeguards Rule; FTC Health Breach Notification Rule), KinPath becomes the only vendor institutional buyers can procure without a lengthy security review — a procurement advantage that compounds as buyers become more privacy-conscious.

### 4.3 The Local Rollout

The plan starts in one metro — Atlanta, Georgia — for two reasons:

- It is the user's home base, giving natural access to local relationships and on-the-ground coordinator recruitment.
- The Atlanta metro has a large, diverse population with meaningful representation of every religious tradition the checklist engine must serve, making it a strong test market for the religion-specific features.

The local rollout proves the consumer and coordinator model, generates the case studies and ROI data needed for B2B pitches, and builds the initial coordinator network density that makes later national expansion credible.

## 5. How to Make It Enticing to Each Buyer

This is the core of the B2B strategy. Each buyer type has a different pain point, a different ROI story, and a different objection set. The pitches below are the actual sales narratives to use.

### 5.1 Employers and HR Teams

**The pain:** Employees who lose a loved one are among the most expensive productivity losses a company has. The Center for Creative Leadership found roughly 30% of grieving employees had to use vacation or sick days after a family death because their employer offered no bereavement support (SHRM). Yet 95% of employees value bereavement-related benefits and 84% of employers plan to expand bereavement support in the coming year (Empathy 2026 Workplace Benefits Report).

**The gap KinPath fills that EAPs do not:** Employee Assistance Programs provide short-term grief counseling, typically three to eight sessions per issue per year, but they do not train managers on grief conversations, do not coordinate leave, do not create return-to-work plans, and do not provide any documented workflow for bereavement cases (Bereave; Bereave). EAPs treat grief as one item on a long list of mental-health issues. KinPath treats the entire death-to-one-year journey as a single managed workflow — the administrative and logistical layer EAPs were never built to handle.

**The ROI story:** KinPath reduces lost productivity by getting employees back to functional capacity faster. The pitch number (a modeling assumption for pilot sales materials, not a researched benchmark): an employee who loses a close family member may lose an estimated 8–12 weeks of productive work time; KinPath's structured coordination and grief-routing targets a 25–40% reduction by removing the administrative chaos that extends grief-related impairment. At a loaded labor cost of $75/hour, recovering even two weeks of productivity per case is worth roughly $6,000 per employee — against a PEPM cost of under $1.15.

**The pitch:** "Your EAP handles the counseling. KinPath handles everything else — the death certificates, the insurance claims, the funeral coordination, the manager communication, the return-to-work plan. One benefit, one PEPM, full coverage of the most disruptive life event your employees will face."

**Pricing:** Under $0.75–$1.15 per employee per month, consistent with the published market-clearing rate (Bereave pricing), with a $1,800 annual minimum. Offer the first 25 employers in the Atlanta market a 60-day free pilot with utilization reporting.

**Objection — "We already have an EAP."** Response: "Your EAP gives employees eight counseling sessions and a hotline. It does not help them file the life insurance claim, find a funeral home, or tell their manager what to say. KinPath is the operational layer underneath your EAP, not a replacement for it — we integrate with your existing EAP, we do not compete with it."

### 5.2 Hospices

**The pain:** Every Medicare-certified hospice must make bereavement services available to the family for up to one year following the patient's death, under a qualified grief professional, and this is surveyed during CMS audits (CMS State Operations Manual, Appendix M; 42 CFR §418.64). This is a federally mandated, unfunded obligation — hospices spend real staff time and money on bereavement follow-up with zero dedicated reimbursement.

**The ROI story:** KinPath converts an unfunded compliance cost into a turnkey, surveyor-ready program. The hospice's bereavement coordinator logs in, sees every family due for contact, sends the outreach, logs the contact, and exports a compliance report for the next CMS survey. The pitch number (a modeling assumption for pilot sales materials, not a researched benchmark): a typical hospice serving 300 deaths per year may spend an estimated 600–900 staff hours per year on bereavement follow-up; KinPath targets a 50–70% reduction through automation and structured workflows, recovering 300–600 staff hours — at $35/hour, that is $10,500–$21,000 in recovered labor cost per year, against a platform fee of a fraction of that.

**The pitch:** "You are already required to do this. KinPath lets you do it in half the staff time, with surveyor-ready documentation, and with a better family experience than phone-call follow-up alone."

**Pricing:** Tiered by annual death volume: under 200 deaths/year, $4,800/year; 200–500, $9,600/year; 500+, $18,000/year. All tiers include the bereavement compliance dashboard and unlimited family activations.

**Objection — "Our families don't want an app."** Response: "They don't have to use it. KinPath's hospice dashboard handles the compliance workflow regardless of family adoption — the coordinator logs contacts, schedules outreach, and exports reports whether or not the family ever opens the family-facing app. Family adoption is a bonus, not a requirement."

### 5.3 Hospitals and Health Systems

**The pain:** Hospital Office of Decedent Affairs teams coordinate final disposition, meet grieving families to review options, and liaise with funeral homes, coroners, and organ procurement organizations (OhioHealth; Duke Health). A study of decedent-affairs programs found the only universal function across surveyed hospitals was answering family questions — a function that is entirely unstructured and unscalable as currently practiced (Archives of Pathology & Laboratory Medicine).

**The ROI story:** KinPath gives the decedent-affairs team a structured workflow tool for exactly the functions they already perform, plus a handoff to the family that continues long after they leave the hospital. The pitch number (a modeling assumption for pilot sales materials, not a researched benchmark): a hospital decedent-affairs coordinator may spend an estimated 45–90 minutes per family on initial disposition guidance; KinPath's structured intake and family handoff targets a reduction to 15–25 minutes by letting the family self-serve the standard steps while the coordinator focuses on exceptions.

**The pitch:** "Your decedent-affairs team answers the same ten questions from every grieving family. KinPath structures those answers, hands the family a guided path that continues for a

full year after they leave your hospital, and gives your team a dashboard that turns ad hoc phone calls into a managed workflow."

**Pricing:** Per-hospital platform fee of $12,000–$24,000/year depending on annual decedent volume, plus per-activation fee for families that continue into the full coordination product.

**Objection — "We can't share patient data with you under HIPAA."** Response: "KinPath is designed to operate in the consumer-directed mode that does not make us a HIPAA business associate — the family downloads the app independently and enters their own information, which HHS guidance confirms does not trigger business-associate status (HHS, Health App Use Scenarios). If you prefer a deeper integration where the hospital pushes records into KinPath, we will sign a BAA and build to the HIPAA Security Rule — but the default referral model requires no PHI sharing at all."

### 5.4 Nursing Homes and Senior Living

**The pain:** Nursing homes and senior living communities are where many deaths occur or are anticipated, and they are the institution with the closest ongoing relationship to the family in the months before death. Yet they have no structured tool for the family handoff at the moment of death, leaving families scrambling.

**The ROI story:** KinPath gives senior living operators a differentiator they can market to prospective residents and families: "When the time comes, we hand your family a guided path — not a brochure and a phone number." This is a genuine sales advantage in a competitive senior-living market where differentiation is otherwise difficult.

**The pitch:** "Offer families something no other senior living community offers: a structured, software-guided handoff for the moment of death and the year that follows. It costs you less per resident per month than a single flower arrangement, and it is a differentiator families will remember when they are choosing where their loved one lives."

**Pricing:** $2–$4 per resident per month, billed to the facility, included as a resident benefit.

### 5.5 Life Insurers

**The pain:** Life insurers process beneficiary claims at the worst possible moment for the family, and claims experience is the single biggest driver of policyholder satisfaction and retention. Empathy built its entire business on this — partnering with MetLife, Voya, Prudential, Principal, and Guardian to deliver bereavement concierge at the point of claim, and growing revenue 10x in two years with 100% client retention (Calcalist; MetLife; Voya).

**The ROI story:** KinPath offers insurers the same claims-experience story as Empathy but with a broader scope — estate and benefits coordination, not just grief support — and with the Benefits Recovery Engine that helps families find and claim policies the insurer might not even know exist. The pitch number: Empathy sells a documented 35% reduction in claims-operations costs to insurers (Empathy); KinPath targets the same cost reduction with a deeper product.

**The pitch:** "Empathy proved insurers will pay for bereavement concierge at the point of claim. KinPath goes further — we help families find policies they didn't know existed, file claims

across every carrier, and manage the entire estate, not just the grief. That is a bigger claims-cost reduction and a better policyholder experience."

**Pricing:** Enterprise contract, negotiated per carrier, structured as a per-claim fee plus a platform license.

### 5.6 EAPs and Benefits Brokers

**The pain:** EAPs are under pressure to expand beyond short-term counseling into the full life-event journey, and benefits brokers are the gatekeepers who decide which voluntary benefits employers adopt. Brokers report that the top reason employers consider a new voluntary benefit is employee demand (60%), followed by recruitment and retention (53%) and affordability (47%) (EBRI).

**The ROI story for brokers:** KinPath is a voluntary benefit that brokers can add to their portfolio with a clear employee-demand story (95% of employees value bereavement benefits), a quantifiable productivity ROI, and a market-clearing price point. It gives brokers a differentiated offering in a category (bereavement) that is the fastest-growing life-event benefit segment, with 84% of employers planning to expand support (Empathy 2026 Workplace Benefits Report).

**The ROI story for EAPs:** KinPath is the operational layer that extends the EAP's counseling into real-world task management — a white-label module the EAP can offer as part of its own platform rather than losing the bereavement category to a standalone vendor.

**The pitch:** "Add KinPath to your benefits stack and give your clients the one benefit their employees will actually use at the worst moment of their lives. It is priced at market-clearing PEPM, it integrates with your existing EAP, and it gives you a differentiated story in the fastest-growing benefits category."

**Pricing:** Broker commission of 15–20% of first-year platform revenue for employer contracts sourced through the broker; EAP white-label licensing negotiated per partner.

### 5.7 Funeral Homes and Vendors

**The pain:** Independent funeral homes (roughly 75% of the 15,000+ U.S. funeral homes) are losing share to chains like SCI and to direct cremation disruptors, and they have no patient-acquisition channel other than reputation and referral.

**The ROI story:** KinPath's vendor marketplace gives independent funeral homes, florists, and caterers a qualified, intent-driven referral pipeline at the exact moment a family is making purchasing decisions — with full disclosure of any listing fees, consistent with FTC Funeral Rule requirements.

**The pitch:** "When a family opens KinPath and selects funeral homes in their area, you appear. You only pay for placement, and the fee is disclosed to the family — which is the FTC-compliant way to do this."

**Pricing:** Free basic listing; paid featured placement at 5–7.5% of referred preneed package value, consistent with disclosed market rates, and only in states that permit referral fees — Washington and New Jersey ban them outright and must be excluded (RCW 18.39.410; NJ Rev. Stat. 45:7-65.3).

## 6. The One-Year Business Plan

This is a month-by-month operating plan for the first 12 months, scoped to the Atlanta, Georgia launch market with the hospice wedge and employer channel as the primary B2B motion.

### Assumptions

- Founding team: the user (Mariesa) as CEO/strategy lead, plus two co-founders or early hires — a head of product/engineering and a head of partnerships/sales.
- Initial capital: $150,000–$250,000 in founder/angel funding, sufficient for 12 months of lean operation in Atlanta.
- Legal entity: Delaware C-corp, with a Georgia foreign-qualification to operate locally.
- Working name: KinPath, pending trademark clearance.

### Month 1: Foundation

- File trademark application for KinPath (and fallback AfterGuide).
- Purchase KinPath domain and brand-squatting variants.
- Incorporate as Delaware C-corp; open business banking.
- Retain a healthcare/privacy attorney to draft the compliance framework: the consumer-directed HIPAA positioning, the GLBA Safeguards Rule program, the FTC Health Breach Notification Rule compliance plan, and the UPL guardrails (conspicuous "not legal advice" notices on every legal-adjacent screen, modeled on the Texas §81.101(c) software safe harbor standard).
- Sign the office lease or co-working space in Atlanta.
- Begin recruiting the head of engineering.

### Month 2: Product Foundation and MVP Scoping

- Lock the MVP scope (see Section 7) and begin building the rules engine for state- and tradition-specific checklists, starting with Georgia law and the four major religious traditions (Jewish, Muslim, Hindu, Catholic).
- Build the document vault with AES-256 encryption at rest and TLS 1.2+ in transit, modeled on the security posture of Trust & Will and Everplans (Trust & Will security; Everplans security).
- Build the family permission model: one Coordinator role with the ability to assign 4–5 helpers, field-level permissions, and the immutable decision log.
- Begin recruiting the first cohort of death doulas for the Atlanta coordinator network — target 10–15 vetted doulas by end of month 3.

### Month 3: Coordinator Network and First Partnerships

- Launch the coordinator network in Atlanta with 10–15 vetted death doulas, each trained on the KinPath platform and certified to KinPath's internal standard (since no state licensure exists for death doulas — NEDA FAQ).
- Begin hospice outreach: identify the 8–10 Medicare-certified hospices in the Atlanta metro and begin relationship-building conversations, targeting 2–3 design-partner agreements by end of month 4.
- Begin employer outreach: identify 15–20 mid-size Atlanta employers (200–2,000 employees) and begin conversations through benefits brokers, targeting 3–5 pilot agreements by end of month 5.
- Soft-launch the consumer MVP to a closed beta of 25–50 Atlanta families, sourced through the coordinator network and hospice relationships.

### Month 4: Hospice Design Partners

- Sign 2–3 hospice design-partner agreements (free or heavily discounted) to pilot the bereavement compliance dashboard.
- Build the hospice dashboard: automated outreach scheduling, contact logging, surveyor-ready compliance reports.
- Onboard the first cohort of families through the hospice channel.
- Begin building the Benefits Recovery Engine MVP: SSA death benefit filing, VA burial benefit filing, life insurance claim tracking.

### Month 5: Employer Pilots and First Revenue

- Sign 3–5 employer pilot agreements (60-day free pilots) through benefits brokers.
- Build the employer dashboard: utilization reporting, leave-status tracking, manager check-in reminders.
- Generate the first case studies and ROI data from the closed beta families.
- Target first revenue: 1–2 employer conversions from pilot to paid at $0.75–$1.15 PEPM.

### Month 6: Market Validation and Seed Round Prep

- Reach 100+ family activations cumulatively across consumer, hospice, and employer channels.
- Compile the first formal ROI report: average benefits recovered per family, average time-to-task-completion, family satisfaction scores, employer utilization.
- Begin seed-round fundraising outreach, targeting $1.5–$3M, with the Atlanta pilot data as the core pitch asset.
- Begin building the partner API layer: Workday integration for employer activation, EAP platform integration discussions.

### Months 7–8: Channel Expansion and Dashboard Hardening

- Expand hospice pipeline from design partners to paying customers — target 5–8 paying hospices by end of month 8.
- Convert employer pilots to paid contracts — target 8–12 paying employers by end of month 8.
- Launch the hospital decedent-affairs dashboard and begin outreach to 2–3 Atlanta hospital systems (Emory, Northside, Wellstar).
- Build the senior living offering and begin outreach to 5–8 Atlanta-area continuing care retirement communities.
- Harden the Benefits Recovery Engine with life insurance carrier integrations and 401(k) claim tracking.

### Months 9–10: Geographic Expansion and Insurer Outreach

- Expand to a second metro — target Nashville or Charlotte, markets with strong hospice density and employer bases.
- Begin life insurer outreach: use the Atlanta ROI data to open conversations with regional and national carriers, targeting 1–2 insurer design-partner agreements.
- Launch the pre-need mode beta to a closed group, beginning to build the recurring-engagement surface and the pre-need-to-at-need data continuity advantage.
- Reach 500+ cumulative family activations.

### Months 11–12: Scale and Series A Prep

- Reach 1,000+ cumulative family activations across 2–3 metros.
- Reach 15–20 paying hospices, 20–30 paying employers, 2–3 hospital systems.
- Close the seed round if not already closed, or begin Series A prep with full year-one operating data.
- Publish the first annual "State of Bereavement Coordination" report — a thought-leadership asset that positions KinPath as the category authority and generates inbound B2B demand.

### KPIs and Revenue Targets

| Metric | Month 6 target | Month 12 target |
|---|---|---|
| Cumulative family activations | 100 | 1,000+ |
| Paying hospices | 3 (design partners) | 15–20 |
| Paying employers | 3–5 (pilots) | 20–30 |
| Hospital systems | 0 | 2–3 |
| Coordinator network (doulas) | 15 | 40–50 |
| Annualized revenue | $50K–$100K | $400K–$700K |

| Metric | Month 6 target | Month 12 target |
|---|---|---|
| Benefits recovered for families (cumulative) | $75K | $500K+ |

The revenue targets are deliberately conservative because year one is a proof-of-concept and channel-building year, not a revenue-maximization year. The strategic priority is signing the right design partners, generating the ROI data, and building the network effects — not maximizing near-term revenue. This mirrors how Empathy, Trust & Will, and Everest all built their businesses: they proved the model with a focused initial channel before scaling.

## 7. MVP Scope: What to Build First and What Not to Build Yet

### Build in the MVP (Months 1–4)

- State- and tradition-specific checklist engine (Georgia + four major religions first)
- Document vault with per-document encryption and access logging
- Family permission model (1 Coordinator + 4–5 helpers, field-level permissions, immutable decision log)
- Shared expenses ledger with splitting
- Vendor directory (curated, manually vetted Atlanta funeral homes, florists, caterers)
- Death doula coordinator matching
- Benefits Recovery Engine MVP (SSA, VA, life insurance claim tracking)
- Consumer-facing mobile app (iOS and Android via a cross-platform framework)
- Hospice bereavement compliance dashboard

### Do not build yet (Months 5+ or later)

- Full partner API integrations (Workday, EAP platforms, carrier claims systems) — begin discussions in month 6, build in months 7–10
- Pre-need mode — beta in months 9–10, full launch in year two
- Hospital decedent-affairs dashboard — build in months 7–8 after hospice dashboard is proven
- Senior living offering — build in months 7–8
- Insurer claims-system integration — pursue only after insurer design partners are signed in months 9–10
- Licensed clinical grief counseling integration — scope as a year-two workstream due to clinical licensing complexity
- National coordinator network expansion beyond Atlanta — only after the Atlanta network proves the unit economics

## 8. Compliance Guardrails for the Expansion

Every feature added in this plan must stay on the compliant side of four regulatory lines:

1. **Do not become an unlicensed funeral arranger.** In states like Florida, "arranging" at-need services is itself a licensed act restricted to funeral directors (Fla. Stat. § 497.372); nine states require hiring a funeral director for at-need arrangements (Funeral Consumers Alliance). KinPath connects families to licensed providers — it does not arrange services itself. Death doulas are unlicensed in every state, which is why the coordinator network is built around them, not around funeral-director-licensed activity (NEDA FAQ; Institute for Justice).
2. **Do not give legal advice.** The Benefits Recovery Engine and the estate/probate checklists provide legal information (deadlines, required documents, official resource links), not legal advice (what action to take in a specific situation). Every legal-adjacent screen carries a persistent, conspicuous notice that it is not a substitute for an attorney, modeled on the Texas §81.101(c) software safe harbor standard, adopted nationally regardless of state requirement (Texas Courts, Legal Information vs. Legal Advice; UPL Committee v. Parsons Technology, 179 F.3d 956).
3. **Avoid HIPAA business-associate status unless deliberately ready for it.** The default partnership model is consumer-directed: the family downloads KinPath independently and enters their own information, which HHS guidance confirms does not make KinPath a HIPAA business associate (HHS, Health App Use Scenarios). Deeper hospital or hospice integrations that involve the facility pushing PHI into KinPath require a signed BAA and HIPAA Security Rule compliance — these are pursued deliberately, not by accident.
4. **Disclose all vendor economics.** Any listing fees, referral fees, or marketplace commissions paid by funeral homes, florists, or caterers are disclosed to families inside the app, consistent with FTC Funeral Rule transparency principles (FTC 2025 compliance guide). Referral fees are not offered in states that ban them (Washington, New Jersey) (RCW 18.39.410; NJ Rev. Stat. 45:7-65.3).

The security baseline remains AES-256 at rest with per-user keys, TLS 1.2+ with perfect forward secrecy in transit, SOC 2 Type II examination, and an explicit commitment never to sell or share user information without consent — the standard already set by Trust & Will and Everplans as the category benchmark (Trust & Will security; Everplans security). This must be built from day one, not retrofitted later.

## 9. The Pitch in One Sentence

For every buyer, the pitch distills to the same sentence: **KinPath is the next-of-kin operating system — the one platform that carries a family from the moment of loss through the one-year anniversary, that employers hand to employees, hospices use for compliance, hospitals use for decedent affairs, and insurers use to improve claims experience.**

That sentence is the company. Everything in this plan — the name, the missing features, the B2B playbooks, the 12-month roadmap — is in service of making that sentence true and making KinPath the only plausible answer when any institution asks, "what do we give families when someone dies?"